

U.S. Medical Devices**U.S. Medtech: Top 8 takeaways from SRS 2026; plus thoughts on JNJ Ottawa clearance and continued ISRG weakness**

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Yesterday, we attended investor day sessions at the first day of SRS 2026, the Society of Robotic Surgery's annual conference. Hot topics included artificial intelligence, the shift to ASCs, new competitors, and telesurgery. We highlight our top 8 takeaways below, and then we share more detailed thoughts from fireside chats with JNJ, MDT and ISRG. Bottom line: Near-term concerns have created an incredibly attractive entry point for ISRG, and stepping back at SRS to consider the longer-term opportunities in the space just serves to deepen our conviction in the ISRG story.

(1) Long runway for growth in soft-tissue robotics. Last week, Intuitive's [2Q26 earnings](#) reignited a debate about the durability of procedure growth in the U.S. market. After the first day of SRS 2026, we came away with a sense that the journey is just beginning for soft-tissue robotic-assisted surgery. Surgeons, hospitals and medtech management teams all highlighted the enormity of the opportunity ahead. We heard an overwhelming sense of excitement about newer applications for soft-tissue robotics (e.g., cardiac, after-hours, benign general, benign GYN, ASCs, endoluminal, and single-port) as well as a recognition that there is more work to be done to fully unlock the opportunity within current applications. We expect to hear more about all these opportunities in the clinical sessions over the remaining days of the conference.

(2) High barriers to entry are getting higher with AI. Competing in soft-tissue robotics is not just about building a competitive piece of equipment that sits in the operating room. Over the last 30 years, Intuitive has spent billions of dollars investing not just in hardware but in a broader ecosystem of value (integrated systems, software, data and insights, workflows, training, service and support, etc.). Listening to hospital leadership teams and influential surgeons at the conference, it's clear to us that these investments have paid off in elevating the basis of competition in the space. While there is openness to considering new technology from JNJ/MDT/others, it's clear that ISRG has set the bar very high—to get real traction, newer platforms will have to solve problems that aren't being solved today. We also heard A LOT of discussion about artificial intelligence can raise the bar even higher by helping hospitals turn data into useful insights, and ISRG is clearly ahead of the pack on this front.

(3) JNJ Ottawa clearance. JNJ reached a major milestone this week with [FDA clearance](#) for Ottawa as the "world's first table-integrated soft-tissue robotic system." Management highlighted Ottawa's (a) 30%-50% smaller footprint, (b) synchronized motion of the table and arms (twin motion), (c) proven Ethicon instruments, (d) Polyphonic open digital ecosystem, and (e) connection to J&J's legacy in continuous education and customer support. JNJ views the global soft-tissue robotic surgery market as being ~8% penetrated, and they see Ottawa as a market-expanding technology. Medtronic has been ahead of JNJ with Hugo clearances in [Europe in Oct 2021](#) and [the U.S. in Dec 2025](#). Nevertheless, given the features and benefits that Ottawa brings to the table, JNJ may ultimately close the gap with MDT despite Hugo's earlier market entry—assuming JNJ can navigate the well-documented execution challenges inherent in launching surgical robotics platforms...

[continued on p3]

BERNSTEIN TICKER TABLE

Ticker	Rating	22 Jul 2026		Price Target	TTM Rel. Perf.	Adjusted EPS				Adjusted P/E (x)		
		Cur	Closing Price			Cur	2025A	2026E	2027E	2025A	2026E	2027E
ISRG (Intuitive Surgical)	O	USD	340.69	685.00	(51.0)%	USD	8.93	11.08	12.71	38.1	30.7	26.8
JNJ (J&J)	M	USD	255.63	261.00	32.3%	USD	10.75	11.65	12.81	23.8	21.9	20.0
MDT (Medtronic)	O	USD	81.89	97.00	(29.8)%	USD	5.53	5.97	6.44	14.8	13.7	12.7
SYK (Stryker)	O	USD	309.24	410.00	(41.7)%	USD	13.66	14.98	16.72	22.6	20.6	18.5
SPX			7,498.96									

O - Outperform, M - Market-Perform, U - Underperform, NR - Not Rated, CS - Coverage Suspended

MDT base year is 2026;

Source: Bloomberg, Bernstein estimates and analysis.

INVESTMENT IMPLICATIONS

We rate ISRG, MDT, and SYK Outperform. We rate JNJ Market-Perform.

Our models can be downloaded here: [ISRG](#) * [JNJ](#) * [MDT](#) * [SYK](#)

RECENT RESEARCH

17 Jul 2026 - [Intuitive Surgical 2Q26: Reviving debates on U.S. growth trend](#)

17 Jul 2026 - [Abbott 2Q26: EPS beat and raise on in-line sales, and a vehement defense of medtech market health](#)

15 Jul 2026 - [Johnson & Johnson 2Q26: Solid beat and raise, reassuring commentary on underlying medtech trends](#)

15 Jul 2026 - [U.S. Medtech: Another step down after HCA pre-release; is there any relief in sight for the sector?](#)

26 Jun 2026 - [Weekend Healthcare Pulse: Deadtech forever? Can Medtech recover?](#)

CONTEXT ON SOFT-TISSUE ROBOTICS PROGRAMS AT MDT AND JNJ

8 Nov 2023 - [Johnson & Johnson: Our key takeaways from the OTTAVA soft tissue robotics update](#)

20 Nov 2020 - [Johnson & Johnson - Med Device analyst day recap: introducing the new "Ottava" robot](#)

25 Sep 2019 - [Medtronic: Our top 10 takeaways from yesterday's robotic-assisted surgery event; everybody wins](#)

6 Sep 2019 - [Intuitive Surgical: What do experts think about J&J's new surgical robot?](#)

15 Aug 2019 - [Intuitive Surgical: What do experts think about Medtronic's new surgical robot?](#)

DETAILS

[continued from p1]

(4) MDT Hugo update. We didn't hear much new news from MDT. Management framed Hugo as a flexible, cost-efficient robotic platform, leveraging modular design, an open console, and integration with MDT's Touch Surgery digital ecosystem, with AI-enabled intraoperative guidance as a key long-term focus. The company continues to pursue a measured U.S. rollout—initially in urology with planned expansion into general surgery and gynecology—while emphasizing bundled offerings and hospital partnerships rather than rapid system placements. Management believes increasing competition will expand robotic adoption from low penetration levels.

(5) SYK continues to be a wildcard. As the leader in hard-tissue orthopedic robotics with Mako, Stryker has made it clear over time that the company sees soft-tissue robotics as an attractive space. Management has also made it clear that Stryker has very strong capabilities in M&A and has been evaluating deals in the soft-tissue robotics space. At the same time, Stryker does not appear to be interested in competing head-to-head with Intuitive. Investors are intrigued about potential deals that could bring an ASC-focused soft-tissue robotics capability to Stryker (e.g., [Distalmotion](#) or [Moon Surgical](#)).

(6) ASCs in the spotlight. There was a lot of discussion about the continued migration of procedures away from hospital inpatient and outpatient settings toward ambulatory surgical centers (ASCs). Robotic platforms can help enable this transition by reducing variability and making procedures more repeatable. ASC customer needs are different (e.g., cost sensitivity, space constraints), so as Intuitive works to scale up in the ASC setting with refurbished Xi systems (XiR), attackers are rushing in with value propositions that are tailored to ASC needs.

(7) Not worried about remanufactured instruments. Surgeon and hospital panelists were frequently asked questions about whether they had tried or were trialing remanufactured instruments. The vast majority were not interested. While there is clearly interest in controlling costs in the hospital, several panelists framed some version of the question: *"if you were performing the procedure on yourself or on your mother, which instruments would you choose?"*

(8) Lots of talk on telesurgery. Telesurgery continues to be a hot topic, and it got plenty of air time during the first day of SRS. As ISRG noted during yesterday's fireside chat and again in their [PR](#) this morning, Intuitive believes telecollaboration technologies can help surgeons learn from one another, provide additional experience to local care teams, and expand access to specialized care. ISRG sees telesurgery as one example of how those capabilities may evolve in the future, and there is potential for telesurgery to address health equity issues globally.

Bottom line: Intuitive's [2Q26 earnings](#) reignited a debate about the durability of U.S. procedure growth. Data points from other medtech/tools players have stoked investor concerns that ISRG may be uniquely exposed to pressures from declining ACA enrollment given the company's procedure mix (i.e., JNJ/ABT did not see any impact from ACA, data points from tools/Dx have been positive). These near-term concerns have created an incredibly attractive entry point for ISRG, and stepping back at SRS to consider the longer-term opportunities in the space just serves to deepen our conviction in the ISRG story.

TAKEAWAYS FROM JNJ/MDT/ISRG

Below, we share more detailed thoughts on SRS 2026 fireside chats with management teams from JNJ, MDT and ISRG.

Johnson & Johnson

J&J announced U.S. clearance for Ottava yesterday, which came 4-5 months earlier than investors had expected, and management took questions during a fireside chat at SRS. IR has scheduled a conference call for investors at 8am ET on August 3 to discuss the announcement.

FDA clearance for Ottava. In a [press release](#) issued yesterday, JNJ announced FDA De Novo authorization for Ottava as the "world's first table-integrated soft-tissue robotic system." JNJ argues that Ottava's architecture—with arms built into the OR table—is designed to compress the OR footprint by roughly 30–50% versus current systems, freeing space and improving workflow, and uses "proven" Ethicon instruments integrated into the platform. The umbrella authorization covers multiple procedures in upper-abdominal general surgery, including Roux-en-Y gastric bypass, gastrectomy, cholecystectomy, splenectomy, gastric sleeve, small bowel resection, appendectomy, lysis of adhesions, fundoplication, and hiatal hernia repair.

JNJ has an ongoing clinical trial to add an inguinal hernia indication for Ottawa.

Key features of Ottawa. In addition to Ottawa's space-saving footprint and connection to JNJ's portfolio of trusted surgical instruments, the company emphasizes Ottawa's open digital ecosystem Polyphonic, which is intended to connect data and systems across sites and support AI-enabled insights. The PR emphasizes simple setup and twin motion, which is the synchronized motion of the table and arms, which can simplify patient repositioning intraoperatively for multi-quadrant access to the anatomy. A [promotional video](#) (click on second image from the top) highlights an "open" philosophy, Ottawa's smaller footprint, "automation from day one," twin motion, proven instrument technology, and an open digital ecosystem. The company also highlights that Ottawa is backed by Johnson & Johnson's legacy in continuous education, with learning resources designed for new and experienced robotic surgeons.

Fireside chat takeaways. In a fireside chat at SRS, Tim Schmid (EVP and Worldwide Chairman of MedTech) and Hani Abouhalka (Company Group Chairman for Surgery) stressed that Ottawa's de novo pathway and lack of a predicate reflect a "truly unique" platform rather than an incremental variant. They stated J&J's ambition to remain a "leader in surgery," extending from open and laparoscopy into robotics, and they expect Ottawa to be a significant contributor to company growth "by the end of the decade." Abouhalka stressed the potential for Polyphonic digital platform to support education, performance analytics, collaborative surgery and further automation, and he highlighted system openness as a differentiator versus more closed incumbents.

Instrumentation. Management did not specify exactly what instrumentation would be available on Ottawa at launch, though Abouhalka did refer to two instruments mentioned in the PR—a two-in-one needle driver designed to reduce unintended suture cutting, and monopolar curved scissors for consistent and complete cuts—as evidence of JNJ's ability to deliver differentiated surgical instruments. From photos and videos posted on JNJ's website, it wasn't clear whether the latest-generation [Dualto Energy System](#) would be available on Ottawa at launch.

Launch details. After Medtronic initiated Hugo launch in South America and Europe before coming to the U.S., J&J will target the U.S. first for Ottawa launch, which the company has positioned as sign of their confidence that the platform is ready to compete. JNJ views the global soft-tissue robotic surgery market as being ~8% penetrated, and they see Ottawa as a market-expanding technology. Launch will be "thoughtful," starting in selected U.S. centers to generate evidence and refine workflows, then expanding more broadly as software, instruments and indications iterate quickly. The team mentioned flexible commercial models and competitive pricing but did not give specifics.

Medtronic

During a fireside chat at SRS, Matt Anderson (SVP and President, Surgical) outlined Medtronic's positioning and progress with the Hugo robotic-assisted surgery platform, emphasizing that increasing competition—highlighted by J&J's recent approval—should expand overall market adoption given robotic penetration remains in the single digits.

Key features of Hugo. Commercially, Medtronic is prioritizing flexible business models (capital sale, leasing, usage-based) and leveraging its large open/laparoscopic portfolio to create bundled value propositions. While pricing specifics were not disclosed, the company continues to position Hugo as a potentially lower-cost-per-procedure alternative over time. Differentiation centers on its modular architecture (offering OR flexibility), open console (enhancing collaboration), and integration with Medtronic's digital Touch Surgery ecosystem. This platform—already deployed in ~1,500 ORs—supports data, analytics, and emerging AI applications, including intraoperative guidance, which management views as a key long-term differentiator versus pure-play robotic systems. This week, MDT issued a [press release](#) highlighting some of the early features of Touch Surgery Aide, the company's next-generation compute platform for the operating room.

Hugo launch status. Medtronic is focused on a measured U.S. rollout, starting in urology and expanding via recently submitted 510(k)s into general surgery and gynecology, which together represent roughly 80% of soft-tissue robotic procedures. Key near-term catalysts include these indication expansions and the introduction of differentiated instruments like LigaSure and ProGrip mesh, alongside a broader ecosystem strategy. Anderson stressed that Hugo is not just a product but a program requiring close hospital partnership, with early U.S. learnings informing a broader launch cadence rather than rapid placement targets.

Longer-term vision. Anderson argued that international experience (35+ countries, tens of thousands of procedures, ~400 publications) has reinforced demand for Medtronic's legacy instruments and validated Hugo's form factor, while also highlighting the importance of training, workflow integration, and ecosystem capabilities. On debated topics, management

acknowledged surgeon preference variability (open vs. immersive console) but reported strong feedback on modularity and workflow flexibility, and downplayed footprint concerns. Anderson noted growing relevance in ambulatory surgery centers and signaled continued investment in robotics platforms tailored to different care settings, with the broader vision of integrating robotics, instruments, and AI into a connected surgical ecosystem.

Intuitive Surgical

Intuitive's presence during the first day of SRS was limited to a pre-recorded fireside chat with CEO Dave Rosa and CFO Jamie Samath (pre-recorded before [2Q26 earnings](#) and before the announcement of JNJ's U.S. clearance for Ottava).

Continuous innovation. Intuitive Surgical management highlighted a shift toward a continuous innovation model, with regular software and hardware updates driven by customer feedback and evaluated through Intuitive's quintuple aim framework: improving outcomes, enhancing patient and care team experience, lowering total cost of care, and expanding access. Recent da Vinci 5 upgrades include [over 100 enhancements](#), notably a digital ruler for intraoperative measurement, surgeon-controlled instrument ejection to improve workflow efficiency, and multi-arm targeting for setup optimization. Cybersecurity was also emphasized as an increasingly important differentiator. On pricing and utilization, Intuitive is extending instrument life to drive manufacturing efficiency and, in certain core instruments, reduce cost per procedure—particularly targeting lower-acuity, cost-sensitive settings such as ambulatory surgery centers (ASCs), where the company sees meaningful growth opportunity.

Innovation priorities. Looking longer term, Rosa framed Intuitive's strategy around building an integrated ecosystem rather than a collection of standalone products, with connectivity, data, and AI as key pillars. While AI remains early, current value is being delivered through digital tools (e.g., My Intuitive + platform), with future potential in intraoperative guidance, automation, and augmented dexterity. Expansion will come via deeper penetration of existing procedures, new indications, and new platforms where Intuitive's capabilities can meaningfully improve clinical and economic outcomes.

Hot topics. In ASCs, adoption is being driven by health systems shifting lower-acuity procedures away from the hospital, with systems like da Vinci XiR positioned to meet cost and workflow needs. On competition (Medtronic Hugo, J&J Ottava, and emerging low-cost international players), management emphasized competing on value and outcomes first, supported by a broad portfolio and flexible economic models. Finally, Rosa positioned telesurgery primarily as a training and proctoring tool rather than a direct care delivery model, aligning with Intuitive's focus on scaling surgical expertise globally. We will hear more about Intuitive's vision for an AI-enabled future of healthcare today at the conference (see this morning's [press release](#) from ISRG).

DISCLOSURE APPENDIX

I. REQUIRED DISCLOSURES

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VALUATION METHODOLOGY**Intuitive Surgical Inc**

We set a price target of \$685 using a P/E multiple of 50x against our forward Q5-Q8 EPS estimate of \$13.66. The P/E target is based on observed absolute and relative historical multiples and our outlook for forward growth. We also use a 30-year DCF analysis as a secondary input to our valuation.

Johnson & Johnson

We set a price target of \$261 using a target P/E multiple of 19.5x against our forward Q5-Q8 EPS estimate of \$13.38. The P/E target reflects observed absolute and relative historical multiples and our outlook for forward growth. We also use current EV/EBITDA vs. history and DCFs as secondary inputs to our valuation.

Medtronic PLC

We set a price target of \$97 using a P/E multiple of 15.0x against our forward Q5-Q8 EPS estimate of \$6.45. The P/E target reflects observed absolute and relative historical multiples and our outlook for forward growth.

Stryker Corp

We set a price target of \$410 using a P/E multiple of 24x against our forward Q5-Q8 EPS estimate of \$17.08. The P/E target reflects observed absolute and relative historical multiples and our outlook for forward growth.

RISKS**Intuitive Surgical Inc**

Downside risks include: Worsening supply chain constraints, major product recall, earlier than expected introduction of surgical robots from major competitors, changes in treatment guidelines for critical procedures (prostatectomy, hysterectomy, etc.), changes to the economic environment that significantly reduce procedure volumes and/or healthcare spending levels, new tariffs on products or components manufactured outside the United States.

Johnson & Johnson

Upside risks include, but are not limited to: stronger than expected earnings growth based on better than expected market conditions (e.g., market-wide improvements in healthcare utilization, volume, or pricing), major disruptions to competitors (e.g., recalls, supply interruptions), or earlier than expected approvals / introductions of key pipeline products.

Downside risks to our target prices include greater than expected margin pressure driven by supply chain constraints and inflation related impact, recalls of major products, FDA warning letters or supply interruptions at major production facilities, accelerated pricing pressure or reimbursement changes in key categories, other policy initiatives or physician guideline changes that may negatively impact product utilization, a rapid deterioration in the global economic environment which could weaken discretionary

healthcare spending, or unexpected fluctuations in foreign currency.

Medtronic PLC

Downside risks include: A major product recall; the risk that key products fail to grow as expected; the risk that the company will overpay for acquisitions to make up for sluggish revenue growth throughout the portfolio; and / or market-wide deceleration across MDT's end markets.

Stryker Corp

Downside risks include: Pressure on hospital capital expenditure budgets; margin headwinds related to inflationary impact and supply chain challenges; a major product recall; the risk that key products fail to grow as expected; the risk that the company will overpay for acquisitions; and / or the risk of a market-wide deceleration across the company's end markets.

RATINGS DEFINITIONS, BENCHMARKS AND DISTRIBUTION

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- Outperform: Stock will outpace the market index by more than 15 pp
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- Underperform: Stock will trail the performance of the market index by more than 15 pp

Coverage Suspended: Coverage of a company under the Bernstein research brand has been suspended. Ratings and price targets are suspended temporarily, are no longer current, and should therefore not be relied upon.

Not Rated: A rating assigned when the stock cannot be accurately valued, or the performance of the company accurately predicted, at the present time. The covering analyst may continue to publish research reports on the company to update investors on events and developments.

Not Covered (NC) denotes companies that are not under coverage.

Bernstein brand stock ratings are based on a 12-month time horizon.

Autonomous brand – common stocks

The Autonomous brand rates common stocks as indicated below. As our benchmarks we use the Bloomberg Europe 600 Financials Price Return Index (E600BK) and Bloomberg Europe Dev Mkt Financials Large and Mid Cap Price Ret Index EUR (EDMFI) index for developed European banks and Payments, the Bloomberg Europe 600 Insurance Price Return Index (E600IN) for European insurers, the S&P 500 and S&P Financials for US banks and Payments coverage, S5LIFE for US Insurance, the S&P Insurance Select Industry (SPSIINS) for US Non-Life Insurers coverage, the Bloomberg Emerging Markets Financials Large, Mid and Small Cap Price Return Index (EMLSF) for emerging market banks and insurers and Payments, and the Bloomberg Japan Financials Large & Mid Cap Price Return Index (JPFILM) for Japanese Financials. Ratings are stated relative to the sector (not the market).

The Autonomous brand has three categories of common stock ratings:

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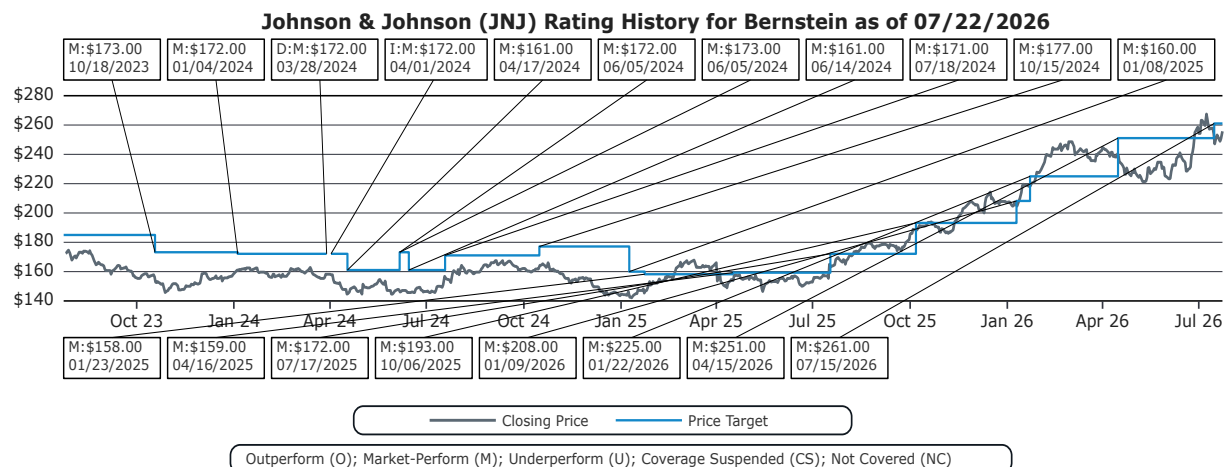
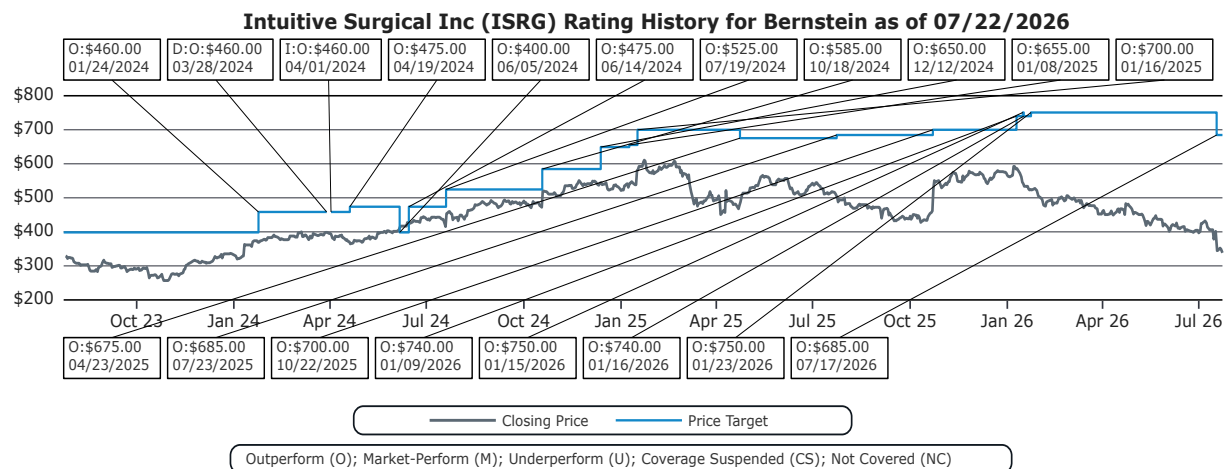
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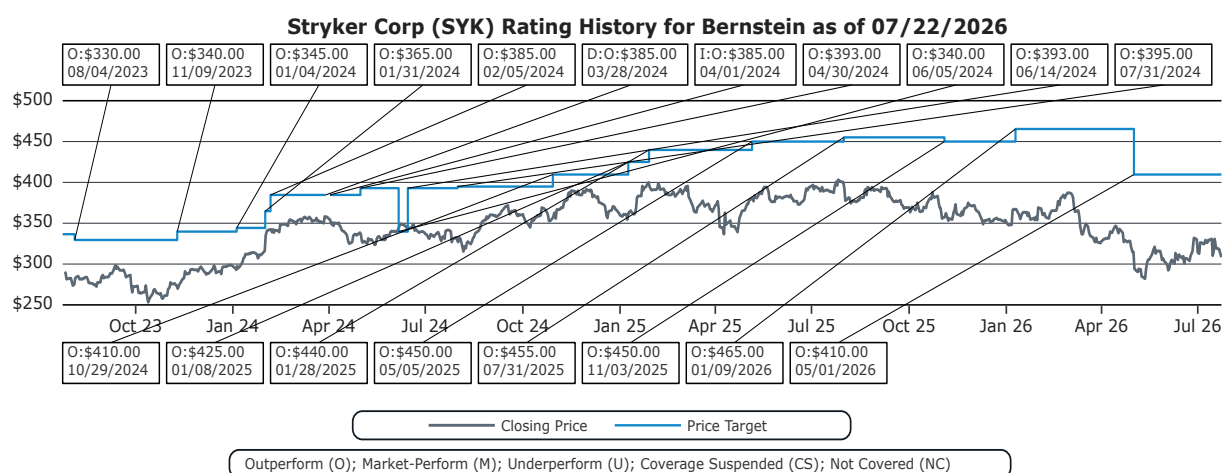
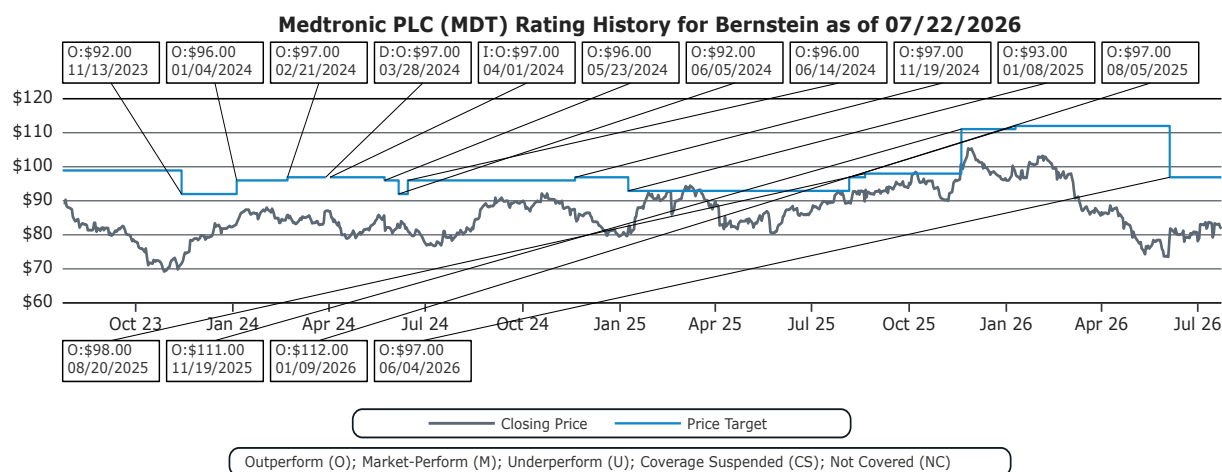
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Outperform	BUY	51.2%	15.3%
Market-Perform (Bernstein Brand) Neutral (Autonomous Brand)	HOLD	35.8%	16.2%
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* These figures represent the percentage of companies within each equity rating category for which affiliates of Bernstein have provided investment banking services within the previous 12 months.
As of June 30, 2026. All figures are updated quarterly.

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Prior to April 1, 2024, Sanford C. Bernstein & Co., LLC. issued the ratings and price target information in the graph(s) below for the following companies: Intuitive Surgical Inc, Johnson & Johnson, Medtronic PLC and Stryker Corp.





All price target and closing price data in the chart(s) above are denominated in the currency noted in the Ticker Table of this report.

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